
Transgender / Gender Dysphoria Interim Payment Framework

Introduction

NHS England commissions specialist Gender Identity Clinics (GICs) for the assessment and treatment of gender dysphoria, but general practitioners (GPs) may be asked to take on the prescribing hormone therapy after a specialist recommendation and provide ongoing care. While GICs provide recommendations, GPs must feel competent and confident to manage hormone treatment, often within a "shared" framework with the specialist clinic.

Some patients may choose to seek gender treatment from independent gender specialists, therefore requests to treat may also come from non-NHS providers.

To support practices who agree to take on the care of these individuals, a transgender/gender dysphoria interim payment framework was set up as a temporary measure while work continued nationally regarding a longer-term solution.

The original framework was launched to practices in January 2020 and has recently been reviewed to streamline the process.

Below describes the framework and process. Practices should note that this is a service framework, not a clinical guideline, so practices are encouraged to familiarise themselves with the regulations and ensure they understand any clinical governance implications.

Application Process for GP practices

Below is an outline of the process for practices who decide to take on prescribing and monitoring responsibilities and request that NHS Devon considers funding this.

1. GP receives a request to prescribe treatment as advised by Gender Identity Specialist (NHS or private)
2. GP to check that they have received all the information they require to undertake the patient care. The gender specialist physician should provide the patient's General Practitioner with the clear written guidance on prescribing and monitoring, be available to provide additional information on request, and answer questions regarding treatment and monitoring at reasonable notice.
3. LMC provided a letter that GPs can use to contact GICs regarding private patients being referred to them, to support them to decide if they feel confident to prescribe required medicines (**attached**)
4. Practices can contact the team at Laurels for non-specific advice (but they cannot give specific advice about patients who haven't been diagnosed by their service). To contact the Laurels, email dpt-tr.TheLaurels@nhs.net with as much information as possible. DPT will endeavour to reply with 2 weeks.

5. GP to complete the Transgender / Gender Dysphoria framework application template using the Microsoft Forms link at [Transgender / Gender Dysphoria Interim Payment Framework Application – Fill in form](#) Please **do not include any patient-identifiable information**.
6. NHS Devon will process the application form, assign a reference number and confirm the funding and any backdated payments to the practice via email. Practices will be advised of how to claim for any approved applications via the Commissioning intelligence portal.

Medication elements within the Framework

As appropriate, practices can claim for one, or both, medication elements detailed below

Element 1

For over 18s only

Medication for masculinisation:

- Testosterone preparations (includes testosterone injections and transdermal gels)

OR

Medication for feminisation:

-Oestrogen preparations (oral and transdermal)

Element 2:

For over 18s only - Gonadotropin releasing hormone analogues

Funding

Element 1 and 2 will be paid at a Tier 3 rate (£132.48 per patient per year).

This mirrors the Specialised medicines service LES payment rate (the tiering framework takes account of the complexity of the drug, the admin and clinical time, tests, unfamiliarity of the drug and the clinical responsibility. (Claims relating to the previous year will be paid at the tier rate for that year).

Claims made in from 1st April 2026 to 31st March 2027 may be backdated a maximum of 12 months according to the treatment start date, subject to the following rules:

Backdating rules for 2026/27

1. The earliest possible backdating date is 1 April 2025.
2. If the treatment start date is after 1 April 2025, the claim will be backdated to the later treatment start date.
3. If the treatment start date is before 1 April 2025, backdating will be limited to 1 April 2025.

Worked examples:

- **Application received:** June 2026
Drug prescribed from: June 2025
Backdating: June 2025 (treatment started after 1 April 2025)

- **Application received:** June 2026
Drug prescribed from: March 2023
Backdating: 1 April 2025 (treatment began before earliest allowable date)

Moving into 2027/28, Claims made in from 1st April 2027 to 31st March 2028 may be backdated a maximum of 12 months according to the treatment start date, subject to the following rules:

Backdating rules for 2027/28

1. The earliest possible backdating date is 1 April 2026.
2. If the treatment start date is after 1 April 2026, the claim will be backdated to the later treatment start date.
3. If the treatment start date is before 1 April 2026, backdating will be limited to 1 April 2026.

Current guidance

GPs may find the following links of use but are encouraged to familiarise themselves with the current national guidance and regulations and ensure that they understand any clinical governance implications.

- [NHS England » Clinical policy: Prescribing of masculinising and feminising hormones for children and adolescents who have gender incongruence or dysphoria – public consultation guide](#)
- [NHS England » Service specification: Gender Identity Services for Adults \(Non-Surgical Interventions\)](#)
- [Trans healthcare - ethical topic - GMC](#)
- <https://www.rcgp.org.uk/representing-you/policy-areas/transgender-care>

Useful Information

To note:

- a. If a patient stops treatment, or has a change to treatment which will impact on the payment rate, practices must notify the ICB of this change
- b. If a patient moves between secondary-care provider, the episode of funding will cease. The GP would then need to complete a second application for funding based on the prescription given by the new secondary-care provider.
- c. If a patient transfers between primary-care practices, then funding would stop at the patient's initial practice, and the receiving GP practice would need to submit a new application for funding.
- d. The principle rationale for points b) and c) is not wanting to assume that provision by practices will continue where there are changes in the arrangements.
- e. The specialist provider list is a national responsibility and unfortunately not yet in place. While NHS Devon can't locally endorse private providers, we will support where we can and keep you informed of progress nationally. Therefore, we would encourage practices to make use of the GP letter for Private or NHS GIC to support them.

- f. As applications must be submitted without any patient identifiable information, practices may wish to create an internal record of funding requests list to help them monitor applications.

Notice Period for Framework

NHS Devon have agreed to set up this payment framework as an interim arrangement whilst a national solution is arrived at. The ICB will continue to review the commissioning arrangements, and if required, practices will be given 3 months notice of any changes.